



Advanced Spine & Orthopedics

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September 10, 2026

Sent via fax to:

Bluebonnet Medicare Advantage — Utilization Management — Organization Determinations

PO Box 0000, Example City, TX 75000

Fax (800) 555-0177

RE: Request for Pre-Service Organization Determination

FIELD	VALUE
Member	Robert L. Castañeda
Date of birth	1955-02-14
Member ID	BBMA-5520-0931
Requested service	L4-L5 decompression with instrumented posterolateral and posterior interbody fusion
Codes	22633, 22840, 22853, 20936
Diagnoses	M43.16, M48.062
Levels	L4-L5
Site of service	Inpatient hospital
Requesting physician	Adaeze Okafor, MD · NPI 1528000025
Facility	Trinity Valley Medical Center · NPI 1740000049
Anticipated date of service	2026-10-06

Dear Utilization Management Reviewer,

On behalf of my patient, Robert L. Castañeda, I am requesting a pre-service organization determination for L4-L5 decompression with instrumented posterolateral and posterior interbody fusion. The service is medically necessary and meets each applicable criterion of Lumbar Spinal Fusion Medical Policy (MP-SPINE-014), as documented below.

1. Request Summary

This request is for L4-L5 decompression with instrumented posterolateral and posterior interbody fusion, planned for 2026-10-06 in the inpatient hospital setting at Trinity Valley Medical Center. It is governed by Lumbar Spinal Fusion Medical Policy (MP-SPINE-014, sample, revised 2026-03).

Every criterion in that policy is addressed in the Criterion-by-Criterion Documentation section. Each supporting record is enclosed and cited by exhibit, so the request can be decided on the documents submitted.

2. Clinical Summary

Mr. Castañeda is a 71-year-old man with grade I degenerative spondylolisthesis at L4-L5 with dynamic instability and severe stenosis, first documented on 2025-10-15. He reports low back pain and left leg pain in an L5 distribution, with neurogenic claudication limiting walking to one block.

Functional impact. He can no longer walk his dog or shop without a motorized cart, and he has given up golf.

ICD-10-CM	DESCRIPTION
M43.16	Spondylolisthesis, lumbar region (primary)
M48.062	Spinal stenosis, lumbar region, with neurogenic claudication

3. Lumbar Fusion Evidence

Validated Outcome Measures

INSTRUMENT	DATE	SCORE	INTERPRETATION	SOURCE
Oswestry Disability Index	2025-12-01	38%	Moderate disability	Exhibit A
Oswestry Disability Index	2026-07-28	52%	Severe disability	Exhibit A

ODI worsened from 38% to 52% despite eight months of conservative care.

Conservative Care

MODALITY	DATES	FREQUENCY / DOSE	RESPONSE	SOURCE
Physical therapy	2025-12-01 to 2026-02-20	18 sessions, flexion-based program	Transient improvement; relapse within weeks	Exhibit B
NSAIDs and gabapentin	2025-10-20 to 2026-07-28	Celecoxib 200 mg daily; gabapentin titrated to 900 mg daily	Partial relief of leg pain	Exhibit B
Transforaminal epidural steroid injections	2026-03-12 to 2026-05-14	Two injections, left L4-L5	Relief lasting under three weeks each time	Exhibit B

Documented conservative care spans 40 weeks (2025-10-20 to 2026-07-28), meeting the policy minimum of 12 weeks, and includes physical therapy, medication management, and injection therapy. Symptoms persisted despite this treatment.

Neurological Examination

The examination on 2026-07-28 (Exhibit A) documents objective neurological deficit corresponding to the compressed levels. Findings include left great toe extension weakness and diminished sensation over the left dorsal foot.

MOTOR		SIDE	GRADE (0-5)
Great toe extension (L5)		Left	4
Ankle dorsiflexion (L4)		Left	5
REFLEX		SIDE	FINDING
Patellar		Bilateral	2+
SENSORY DISTRIBUTION		SIDE	FINDING
L5 dermatome		Left	Diminished to pinprick

Imaging and Clinical Concordance

STUDY	DATE	LEVEL	FINDING	SOURCE
MRI lumbar spine	2026-04-15	L4-L5	Grade I anterolisthesis with severe central and left lateral recess stenosis	Exhibit C
Flexion-extension radiographs	2026-04-15	L4-L5	4.5 mm sagittal translation between flexion and extension	Exhibit D

Each treated level correlates with the examination and the patient's symptom distribution:

LEVEL	IMAGING FINDING	EXAMINATION FINDING	SYMPTOM CORRELATION
L4-L5	Severe stenosis, left lateral recess	Left EHL 4/5, left L5 sensory loss	Left L5 radicular pain, claudication

The most recent advanced imaging (MRI lumbar spine, 2026-04-15) was obtained 5 months before the anticipated date of service, within the policy's 12-month window.

Fusion Indication

Indication: Degenerative spondylolisthesis with dynamic instability.

MEASUREMENT	VALUE	DATE	SOURCE
Sagittal translation, flexion to extension	4.5 mm	2026-04-15	Exhibit D
Slip grade (Meyerding)	Grade I (18%)	2026-04-15	Exhibit D

Why decompression alone is inadequate. Adequate decompression of the left lateral recess requires partial facetectomy at a segment that already translates 4.5 mm. Decompression alone would further destabilize the segment and risk slip progression and recurrent stenosis.

Modifiable Risk Factors

FACTOR	FINDING	DATE	POLICY THRESHOLD	SOURCE
Nicotine	Former, quit 2026-05-01; urine cotinine negative on 2026-08-20	2026-08-20	Nicotine-free ≥ 6 weeks	Exhibit E
BMI	32.1 kg/m²	2026-07-28	≤ 40	Exhibit A
HbA1c	6.9%	2026-08-20	≤ 8.0%	Exhibit E

Level Selection

1 level requested: L4-L5. This is within the policy limit of 2 levels per procedure.

LEVEL	RATIONALE	SOURCE
L4-L5	Grade I degenerative spondylolisthesis with 4.5 mm dynamic translation and severe stenosis; concordant L5 deficit.	Exhibit C

Adjacent levels not included

- **L3-L4:** Mild disc bulge without stenosis or instability.

Instrumentation, Devices, and Graft

CODE	DESCRIPTION	UNITS	CLINICAL JUSTIFICATION
22840	Posterior non-segmental instrumentation	1	Bilateral L4–L5 pedicle screw fixation stabilizes the dynamically unstable segment.
22853	Insertion of interbody biomechanical device, each interspace	1	An interbody cage restores disc height and indirectly decompresses the foramina.
20936	Autograft, local, same incision, for spine surgery	1	Local autograft from the decompression is used for arthrodesis, avoiding a separate harvest site.

Site of Service

Inpatient admission is requested. Instrumented fusion in a 71-year-old with type 2 diabetes requires inpatient care.

- Expected stay spans at least two midnights for pain control, mobilization, and drain management.
- Diabetes requires perioperative glucose management.

4. Criterion-by-Criterion Documentation

Each criterion below is quoted from Lumbar Spinal Fusion Medical Policy (MP-SPINE-014).

#	CRITERION	STATUS	SUPPORTING DOCUMENTATION	SOURCE
C1	Spondylolisthesis with documented instability on flexion-extension imaging	Met	4.5 mm translation on flexion-extension radiographs	Exhibit D
C2	At least 12 weeks of conservative care including physical therapy and medication	Met	PT, medication, and two epidural injections from 2025-10-20 to 2026-07-28	Exhibit B
C3	Neurological deficit or neurogenic claudication concordant with imaging	Met	Left L5 motor and sensory deficit; claudication at one block	Exhibit A

#	CRITERION	STATUS	SUPPORTING DOCUMENTATION	SOURCE
C4	Verified nicotine abstinence for at least 6 weeks before surgery	Met	Quit 2026-05-01; urine cotinine negative 2026-08-20	Exhibit E
C5	BMI and glycemic control within surgical safety thresholds	Met	BMI 32.1; HbA1c 6.9%	Exhibit E

5. Medical Necessity

Instrumented fusion at L4-L5 addresses both the neural compression causing his L5 deficit and claudication and the dynamic instability that makes decompression alone unsafe.

Alternatives considered. Decompression alone risks iatrogenic instability and slip progression. Further injections have produced diminishing, short-lived relief.

Consequences of delay or denial. Progressive motor deficit, further loss of walking tolerance, and deconditioning that raises surgical risk later.

6. Submission Notes

- Please issue the determination within the Medicare Advantage timeframe that applies under CMS-0057-F: 7 calendar days for a standard request. Please send written notice to the member and to this office.
- Where Medicare coverage criteria are fully established for this service, 42 C.F.R. § 422.101(b)(6) requires the plan to apply them. If the plan applies internal criteria instead, please identify them and the publicly available evidence supporting them.
- If this request cannot be approved as submitted, please arrange a peer-to-peer discussion with a clinician in orthopedic spine surgery or a closely related specialty before an adverse determination is issued.

7. Requested Action

Please authorize the following services:

CODE	MODIFIERS	DESCRIPTION	UNITS
22633	—	Combined posterolateral and posterior interbody arthrodesis, lumbar, single interspace	1
22840	—	Posterior non-segmental instrumentation	1
22853	—	Insertion of interbody biomechanical device, each interspace	1
20936	—	Autograft, local, same incision, for spine surgery	1

Site of service: Inpatient hospital, Trinity Valley Medical Center. Anticipated date of service: 2026-10-06.

I am available for a peer-to-peer discussion at (817) 555-0144 (weekdays 7:00–9:00 a.m. and 12:00–1:00 p.m. Central). Questions about this request may be directed to Prior Authorization Coordinator at (817) 555-0145 or priorauth@example-spine.test.

Thank you for your prompt review.

Sincerely,

Adaeze Okafor, MD

Orthopedic Spine Surgery · NPI 1528000025

Electronically affirmed by Adaeze Okafor, MD on 2026-09-10 09:15 CT. I attest that the information in this letter is accurate and supported by the enclosed medical records.

Enclosures

EXHIBIT	DOCUMENT	DATE	PAGES
A	Office notes with neurological examination and ODI	2025-12-01 to 2026-07-28	8
B	Physical therapy, medication, and injection records	2025-10-20 to 2026-07-28	14
C	MRI lumbar spine report	2026-04-15	2
D	Flexion-extension radiograph report	2026-04-15	1
E	Laboratory results: urine cotinine and HbA1c	2026-08-20	2

cc: Robert L. Castañeda